Akoizo Original Practice - Form 3

Division B | Disease Detectives | 50-minute written practice

Original Akoizo curriculum practice. This is not a historical tournament paper. Written build, CAD, lab and communication exercises supplement real practical preparation.

Name: _____ Date: _____

1. (2 points) A line list usually has one row per…

Unit: Outbreak investigation and descriptive evidence.
Worked-case data: Six cases have onset on day 1, eight on day 2, and two on day 3. Their report dates are all day 5.

A. Case or explicitly defined event

B. Map color

C. Hypothesis only

D. Calendar month necessarily

Answer: _____

2. (2 points) Directly contacting facilities for case records is…

Unit: Surveillance and the public health information cycle.
Worked-case data: Reported cases double in the month a new laboratory and more testing sites begin reporting.

A. Active surveillance

B. Only passive reporting

C. A randomized trial

D. A case-control definition

Answer: _____

3. (2 points) Rehabilitation after established disability is usually…

Unit: Levels of prevention and population strategies.
Worked-case data: In 1000 synthetic people, 10 have a condition. A test has sensitivity 90% and specificity 90%. Calculate expected counts and PPV.

A. Tertiary prevention

B. Primary prevention

C. A new incidence definition

D. A case-control selection rule

Answer: _____

4. (2 points) What can create an artificial report-date spike?

Unit: Outbreak investigation and descriptive evidence.
Worked-case data: Six cases have onset on day 1, eight on day 2, and two on day 3. Their report dates are all day 5.

A. Batch reporting

B. Perfect onset data

C. A uniform denominator

D. Removing duplicate cases

Answer: _____

5. (2 points) A selected network of sites is…

Unit: Surveillance and the public health information cycle.
Worked-case data: Reported cases double in the month a new laboratory and more testing sites begin reporting.

A. Sentinel surveillance

B. A complete census necessarily

C. A perfect random sample automatically

D. No surveillance

Answer: _____

6. (2 points) Why can PPV be low despite high specificity?

Unit: Levels of prevention and population strategies.
Worked-case data: In 1000 synthetic people, 10 have a condition. A test has sensitivity 90% and specificity 90%. Calculate expected counts and PPV.

A. Low prevalence can leave many false positives

B. Sensitivity equals prevalence

C. All tests are random

D. PPV ignores false positives

Answer: _____

7. (2 points) A spot map directly supplies…

Unit: Outbreak investigation and descriptive evidence.
Worked-case data: Six cases have onset on day 1, eight on day 2, and two on day 3. Their report dates are all day 5.

A. Case locations

B. Population-adjusted risk automatically

C. The causal agent

D. Complete exposure histories

Answer: _____

8. (2 points) A broad syndrome signal often trades specificity for…

Unit: Surveillance and the public health information cycle.
Worked-case data: Reported cases double in the month a new laboratory and more testing sites begin reporting.

A. Timeliness or sensitivity

B. Guaranteed causality

C. Perfect diagnosis

D. Elimination of false alarms

Answer: _____

9. (2 points) A safe-water intervention before illness is…

Unit: Levels of prevention and population strategies.
Worked-case data: In 1000 synthetic people, 10 have a condition. A test has sensitivity 90% and specificity 90%. Calculate expected counts and PPV.

A. Primary prevention

B. Tertiary prevention necessarily

C. Only surveillance

D. A diagnostic test

Answer: _____

10. (4 points) Give three useful line-list fields.

Unit: Outbreak investigation and descriptive evidence.
Worked-case data: Six cases have onset on day 1, eight on day 2, and two on day 3. Their report dates are all day 5.

Answer: _____
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_____

11. (4 points) List four steps in a surveillance cycle.

Unit: Surveillance and the public health information cycle.
Worked-case data: Reported cases double in the month a new laboratory and more testing sites begin reporting.

Answer: _____
_____
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12. (4 points) Calculate expected false positives in the example.

Unit: Levels of prevention and population strategies.
Worked-case data: In 1000 synthetic people, 10 have a condition. A test has sensitivity 90% and specificity 90%. Calculate expected counts and PPV.

Answer: _____
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13. (4 points) Why vary epidemic-curve bin width?

Unit: Outbreak investigation and descriptive evidence.
Worked-case data: Six cases have onset on day 1, eight on day 2, and two on day 3. Their report dates are all day 5.

Answer: _____
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14. (4 points) Why inspect testing changes?

Unit: Surveillance and the public health information cycle.
Worked-case data: Reported cases double in the month a new laboratory and more testing sites begin reporting.

Answer: _____
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_____

15. (4 points) Calculate PPV.

Unit: Levels of prevention and population strategies.
Worked-case data: In 1000 synthetic people, 10 have a condition. A test has sensitivity 90% and specificity 90%. Calculate expected counts and PPV.

Answer: _____
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16. (4 points) Why exclude suspected exposure from the outcome definition?

Unit: Outbreak investigation and descriptive evidence.
Worked-case data: Six cases have onset on day 1, eight on day 2, and two on day 3. Their report dates are all day 5.

Answer: _____
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17. (4 points) What does representativeness concern?

Unit: Surveillance and the public health information cycle.
Worked-case data: Reported cases double in the month a new laboratory and more testing sites begin reporting.

Answer: _____
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_____

18. (4 points) Why is finding more cases not automatically a benefit?

Unit: Levels of prevention and population strategies.
Worked-case data: In 1000 synthetic people, 10 have a condition. A test has sensitivity 90% and specificity 90%. Calculate expected counts and PPV.

Answer: _____
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